

Controlled reviewed preview. Not for clinical use.

This exact substantive candidate passed independent clinical review and pharmacy verification; reviewer identities are retained privately. It remains a controlled preview with no publication authority until the complete release family is generated, verified and its exact production hashes are ratified. The published guideline remains v2.0.

An independent three-track audit on 30 July 2026 corrected several factual errors in this draft, including two where an earlier revision wrongly withdrew a correct v2.0 statement. Those corrections were included in the exact candidate that passed independent clinical review and pharmacy verification. Publication authority remains false for this preview pending complete release-family verification and exact production-hash ratification.

Every statement of marketing authorisation, HTA recommendation and commissioning status below must be re-checked against the official source before ratification. Where a v2.0 statement could not be reproduced against the live source on 30 July 2026, the change is marked with a **changed since v2.0** block giving the reason.

Publication model: England access framework with separate devolved-nation notes. United Kingdom and non-United Kingdom positions are demarcated throughout: blocks headed **outside the United Kingdom** describe evidence or regulatory status in other jurisdictions and *never* establish a route to treatment for a patient in the United Kingdom.

MCL v2.1 quick reference

One-page quick reference

The tables that follow this summary give the reasoning. This one is the compressed form, intended to be printed and taken to clinic or MDT. Certainty and access are recorded separately and deliberately: they answer different questions and they frequently disagree.

Certainty A to E and access class R1 to R5 are defined immediately below. Priority P1 to P4 is the practical instruction. Draft, not for clinical use — confirm every access statement against the live source.

Setting

Option

Certainty

England access

Priority

First line, transplant-suitable

Cytarabine induction, ASCT, rituximab maintenance 3 years

R3 guideline-supported

P1

TRIANGLE ibrutinib regimen

R4 licensed, company scheme

P3

Rituximab maintenance in a TRIANGLE pathway

R3

P2

Obinutuzumab substituted for rituximab

R5 no licence

P4

First line, older or transplant-ineligible

Bendamustine–rituximab, then maintenance ≥2 years

R1 routine — policy 17088P, off-label

P1

VR-CAP

R1 routine — TA370

P2

R-BAC

C

R3 guideline-supported

P2

Acalabrutinib with bendamustine–rituximab

R4 licensed, company scheme

P3

Adding bortezomib to BR, or lenalidomide to maintenance

—

P4

Ibrutinib with BR (SHINE), or ibrutinib–rituximab (ENRICH)

R5 no route

P4

TP53-mutated

Trial referral

R3

P1

Anti-CD20 with BTK and BCL2 inhibition, first line

C

R5 no licence

P4

MRD-guided cessation

R5

P4

Relapsed or refractory

Ibrutinib or zanubrutinib after exactly one line

R1 routine — TA502, TA1081

P2

Brexucabtagene autoleucel after ≥2 lines incl. BTKi

C

R2 CDF — review remitted after appeal

P2

Pirtobrutinib after covalent BTKi

C

R4 licensed, no route

P3

Lisocabtagene maraleucel

C

R4 licensed, no route

P3

Lenalidomide monotherapy

C

R4 licensed, no route

P3

Ibrutinib with venetoclax

R5 no MCL licence

P4

Glofitamab — GLOBRYTE trial

C

R5 trial only

P4

Sonrotoclax — CELESTIAL-RRMCL trial

C

R5 trial only

P4

Allogeneic HCT after CAR-T failure

R3 specialist decision

P3

THREE LINES TO CARRY IN YOUR HEAD

Bendamustine–rituximab is routinely commissioned in first line and is *off-label* — NHS England policy 17088P says so itself. Commissioning and licensing are not the same thing and can point in opposite directions.

The two first-line company schemes divide on transplant eligibility, in opposite directions: acalabrutinib–bendamustine–rituximab is licensed for patients **not** eligible for ASCT, the TRIANGLE regimen for those who **are**.

Brexucabtagene autoleucel is the only cellular therapy with an England route, TA677 managed access remains live, while the post-managed-access review remains unresolved following remittal. Confirm before you promise.

Discuss allogeneic transplantation *before* CAR-T, at covalent BTK-inhibitor progression. Remission status at transplant drives the outcome, and a patient who has already failed CAR-T is usually neither in remission nor fit enough.